BASELINE → TREATMENT → FOLLOW-UP

Safety tests, symptoms and response review

The team books and reviews each test. Mandatory medicine-safety checks are distinct from individually timed MRI and tumour-tissue tests. There is no universal blood tumour marker for these treatment cycles.

Check	Before starting	During treatment	Between visits / recovery	Next-cycle / response review	After a pause or stop
Blood count / kidneys / liver	FBC, renal/liver profile, glucose, HBsAg and anti-HBc	FBC weekly during six-week RT; kidneys/liver before week 4	Continue arranged checks and recovery review before adjuvant treatment	Day 1 FBC/renal/LFT each cycle; Day 22 FBC under EMA guidance	LFT after cycles; follow delayed toxicity after the last dose
Neurology / medicines / function	Record neurological symptoms, daily function and current medicines	Review seizures, headache, cognition, mood, walking and any steroid plan	Report changes early; do not wait for the next MRI	Review treatment tolerance, medication support and patient goals	Agree follow-up and rehabilitation or symptom support
MRI / diagnosis	Confirm integrated diagnosis, molecular results and baseline imaging	MRI timing according to the neuro-oncology plan	Keep the booking and result-review contact in your passport	Interpret scan with symptoms, treatment effects and clinical review	Agree the next imaging and clinical review

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00334 v4 · current Irish regimen · NCCN Guidelines for Patients: Brain Cancer Glioma, 2026 · patient education reference · EMA Temodal product information and patient leaflet

THE WHOLE TREATMENT JOURNEY

Temozolomide with radiotherapy, then adjuvant temozolomide: treatment and checks

Adults with newly diagnosed glioblastoma under NCCP 00334.

Take temozolomide daily throughout six weeks of radiotherapy, including weekends as prescribed. After a four-week break, take temozolomide on Days 1–5 of each 28-day cycle for up to six adjuvant cycles.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	During treatment	Between visits / recovery	Next-cycle / response review	After a pause or stop
Blood count / kidneys / liver NCCP 00334	FBC, renal/liver profile, glucose, HBsAg and anti-HBc	FBC weekly during six-week RT; kidneys/liver before week 4	Continue arranged checks and recovery review before adjuvant treatment	Day 1 FBC/renal/LFT each cycle; Day 22 FBC under EMA guidance	LFT after cycles; follow delayed toxicity after the last dose
Neurology / medicines / function NCCP 00334	Record neurological symptoms, daily function and current medicines	Review seizures, headache, cognition, mood, walking and any steroid plan	Report changes early; do not wait for the next MRI	Review treatment tolerance, medication support and patient goals	Agree follow-up and rehabilitation or symptom support
MRI / diagnosis NCCP 00334	Confirm integrated diagnosis, molecular results and baseline imaging	MRI timing according to the neuro-oncology plan	Keep the booking and result-review contact in your passport	Interpret scan with symptoms, treatment effects and clinical review	Agree the next imaging and clinical review

What each check is for

1 Temozolomide with six weeks of radiotherapy

Daily capsule phase

This treatment combines daily temozolomide with radiotherapy, followed by a break and further temozolomide cycles, to help control newly diagnosed glioblastoma. Your team discusses the expected benefit and how it fits after surgery or biopsy.

Ask / act: Confirm the current phase, prescription, blood tests and support medicines.

Sources: NCCP 00334

2 Four-week break before adjuvant treatment

Recovery and review

This treatment combines daily temozolomide with radiotherapy, followed by a break and further temozolomide cycles, to help control newly diagnosed glioblastoma. Your team discusses the expected benefit and how it fits after surgery or biopsy.

Ask / act: Confirm the current phase, prescription, blood tests and support medicines.

Sources: NCCP 00334

3 Up to six adjuvant 28-day cycles

Five capsule days per cycle

This treatment combines daily temozolomide with radiotherapy, followed by a break and further temozolomide cycles, to help control newly diagnosed glioblastoma. Your team discusses the expected benefit and how it fits after surgery or biopsy.

Ask / act: Confirm the current phase, prescription, blood tests and support medicines.

Sources: NCCP 00334

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Sources checked 2026-11-05 · Source reconciled; independent clinical, pharmacy and patient/PPI review pending · SACTCheck v0.84.38